

العمر(تاريخ الميلاد) : 07-01-1979 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التعليق على التشخيص : -----

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

التشخيص النهائي :
 خطة العلاج :
 المستشفى : قسم :
 جهة العلاج : نفقات :
 ملاحظات :

رئيس
المجلس الطبي المتخصص

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Name: نعمه عبد النبي احمد عطيه National ID: 27901071303348	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Goserelin (Zoladex) subcutaneous

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٠٢٤/١٢ ٢٧٩.٠١.٧١٣.٠٣٣:٨

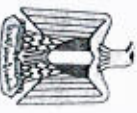
حاصلة على مذهب قس صفاني

متروجة

التي مسلمة

احمد عبدالله ابراهيم غريب

البطاقة سارية حتى ٢٠٣١/١٢/٠٨



DMS
1000



جواز سفر فلسطيني
الاسم: محمد علي محمد

نصفه

عبد النبي احمد عطيه

٢٧٩.٠١.٧١٣.٠٣٣:٨

شيرا الخيمة ثان - القنوية

٢٧٩.٠١.٧١٣.٠٣٣:٨

KX4438545

MDT

Patient Name	نعمه عبدالنبي	Age	46
Sex	f	Special habits	
Family History		Comorbidity	
Surgical History		Telephone	01110722876
Menopausal Status	pre	Marital Status	
ID	27901071303348	Medical ID	1841/24
Date	19/10/2025	Reason for MDT	

Medical Report

Affected breast: left

Pathology:

- IDC
- Er 7/8
- Pr 7/8
- Her2 3+

cT4b N1 M0

S.M 10/2025

- Rt BIRADS 1
- Lt free operative bed
-

Metsworkup:

- PAUS 10/2025: Free
- TVUS 10/2025: Free

Doctor's notes:

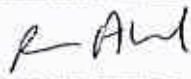
- PS0
- 1/2025 LT MRM pT0 pN0/3
- 2/2025 started tamox
- 9/2025 ended ltp
- E2 28 / FSH 43

Recommendations:

- Possibility to shift to AI

MDT DECISION

For: AI + zoladex x 7 years

Name	حنان عزت	رانيا شفيق	احمد نجيب
Signature			



Patient Name : **الاستاذة / د. سميرة محمد عطية**
Patient No. : 27901071303348
Path No. : 42/25
Sex : Female
Age : 45 Y
Referring Doc :

Request No. : 13001198
Request Date : 04/01/2025
Verification Date : 16/01/2025
Financial Category : **دوريات / مستلزم - مدفوع**
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, post NACT.

Nature Specimen: Left modified radical mastectomy specimen.

Gross Examination:

- Procedure: Left MRM measured 25x18x5.5 cm with covering skin ellipse measured 19x16 cm.
 - Tumor extent: An ill defined rubbery greyish white tumor bed site.
 - Size: 5.5x5x2 cm.
 - Skin involvement: Nipple & areola were unremarkable grossly.
 - Distance from margins: 1 cm away from deep margin and 2 cm away from skin.
 - Lymph nodes number and size: After careful repeated serialing of attached axillary tail, it entangled 3 LNs measured up to 2.5x1.5 cm.
 - Designation:
 - 1-5: Tumor bed site.
 - 6: deep margin.
 - 7: Nipple.
 - 8: Lymph nodes.
- Dissected by: Dr. Samia Ali.

Microscopic Examination:

- Histological type: No residual invasive tumor.
- Ductal carcinoma in situ: Not detected.
- Skin involvement: Free.
- Angiovascular invasion: Not detected.
- Perineural invasion: Not detected.
- Lymph node involvement: Free 0/3 with therapy effect
- Resection Margins: Free deep margin.
- Histologic response to therapy: Complete pathological response.

Verified by :

Dr. Toka Medhat

Printing Date : 28/01/2025 13:26:08

Page 1 of 2

Printing By : marwa mohamed youss



جولة رقم (4) - الدور الأول - مبنى رقم (987) - الدور الثاني - مبنى القاعة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mar El Qadima - Cairo, Egypt

Patient Name : (الاسم) / Walaa Ghoniem
Patient No. : 27901071303348
Path No. : 42/25
Sex : Female
Age : 45 Y
Referring Doc :

Request No. : 13001198
Request Date : 04/01/2025
Verification Date : 16/01/2025
Financial Category : Out Patient
Department :
Registered by : marwa mohamed youss

Pathology Report

* Microcalcifications: Not detected.

Diagnosis:

- Left breast cancer post NACT, MRM specimen.
- No residual invasive tumor.
- Free deep surgical resection margin.
- Free skin.
- Free lymph nodes (0/3) with therapy effect.
- Complete pathological response to therapy.
- Stage: ypT0, ypN0.

Signature
Dr. Walaa Ghoniem
Dr. Yasmine Essam

Verified by :
Dr. Toka Medhat

Printing Date : 28-01-2025 11:26:08

Page 2 of 2

Printing By : marwa mohamed youss

غرفة رقم (1) - الدور الأول - مبنى رقم (987) - لورنيزان النيل - ممر المشاة - القاهرة - مصر
Room No. (1) - First Floor Building No. 987 - Nile Corniche - Mitr El Qadima - Cairo, Egypt



Patient Name: [Name]
Patient ID: 276010712024
Path No: 2133/24
Sex: Female
Age: 45 Y
Referring Doctor: [Name]

Request No: [Number]
Request Date: 27/06/2024
Verification Date: 01/07/2024
Financial Category: [Category]
Department: [Department]
Hospital and City: [Hospital and City]
Out Patient: [Status]

Chief Complaint:
Left breast mass.

Pathology Report

Nature of specimen:
Incisional biopsy, collected at 27/06/2024, 10:30 am.

Gross examination:
Multiple grayish white lesions cover up to 1.7 cm, totally submitted.
HIC technique. Sections prepared from paraffin block for immunohistochemistry using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4-4) IgG clone (30-4).

- Microscopic examination
- Histological type: Invasive duct carcinoma
- Grade (differentiation, nuclear pleomorphism, mitosis): (3+2+2) Grade II
- DCIS: focal
- Angiolymphatic invasion: Couldn't be detected in the received biopsy
- Perineural invasion: No
- IHC (Positive control showed appropriate reactivity):
 - ER: Strong nuclear reaction in 100% tumor cells (Allred score 7/8)
 - PR: Moderate nuclear reaction in 80% tumor cells (Allred score 7/8)
 - HER2: Strong complete membrane reaction in > 10% of tumor cells (score 3+)
 - Ki67: 40% nuclear reaction of tumor cells

Diagnosis:
Left breast mass, Incisional biopsy.

Verified by:
Dr. Hiba Hasan

Reporting Date: 01/07/2024 12:30:02

Page 1 of 2

Printing By: [Name]



Report No: [Number]
Request No: [Number]
Patient Name: [Name]
Referring Doctor: [Name]
Hospital and City: [Hospital and City]



Patient Name: No'ma A. Naby Ahmed Altoya	Date : 10/02/2025
Exam : C.T CHEST	Age : 46 Year(s)
Medical ID: 18411/2024	Pat. ID: 53908

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -

- ↓ **Case of left MRM.**
- ↓ **Compared to study dated 9-10-2024 current one revealed:**
- Case of left MRM with operative bed overlying diffuse skin thickening and left axillary nodal lesion measures about 10x11 mm for further us correlation.
- Still noted Few prominent left lower deep cervical ,supraclavicular and internal mammary lymph nodes.
- Currently appreciated right lower lung lobe few subcentemetric focal pleural based nodularities for follow up.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- no sizable masses, consolidation or cavitations.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.
- Stationary course regarding right hepatic lobe segment VII hypodense focal lesion measuring 1 cm.

OPINION:

- Case of left MRM with operative bed overlying diffuse skin thickening and left axillary nodal lesion for further us correlation.
- Currently appreciated right lower lung lobe few pleural based nodularities follow up.
- Stationary course left of the study.

Dr. Sarah Magdy

Prof. Menna Taffah Hanafy, M.D.



T. T. / 2 / 11 : 6-30

المعلم: عبد النبي احمد

C. T. R E P O R T

Multislice CT scan examination was done for the Chest, Abdomen & Pelvis. Multiplanar reconstruction images were obtained following IV contrast enhancement and gut opacification with Telebriz.

- Prominent pulmonary parenchymal vessels has been shown with bronchial wall thickening basal fine reticulations as well as atelectatic bands mainly basal.
- Pleural and subpleural small soft tissue nodules are also noted at the right upper lung zone posteriorly and in the left lung.

Otherwise, no other pulmonary abnormality could be detected.

The pleural recesses are clear.

No mediastinal mass lesion or gross lymphadenopathy could be detected.

Left breast soft tissue density is also seen.

- Right hepatic lobe superior segment small non-opacified hypodense lesion is seen (about 2 mm)
- The colon is also mildly distended, however, the para colic fat as well as peritoneal recess are preserved.
- Otherwise, the rest of the abdominal and pelvic organs show no gross abnormality.
- No ascites or lymphadenopathy could be detected.

Impression:

Impression:

- * Mild interstitial pulmonary reaction has been shown with reticulo nodular reaction (? Inflammatory) (? Others).
- * Right hepatic lobe small focal hypodense lesion also noted.
- * Clinical correlation would be recommended if disseminated neoplastic disease (metastatic) has to be further evaluated.

YOURS SINCERELY,
Prof. Dr. Mohammed Fouad,
PROF. OF RADIO PHYSICS
CAIRO UNIVERSITY



Dr. El Salam Cancer Hospital
Radiology Department
Cairo, Egypt

Patient Name: No'ma A. Naby Ahmad Attaya	Date : 23/08/2024
Age : 45 Year(s)	Patient ID: 53908
Exam : TRIPHASIC ABDOMEN & PELVIS	ID: 1841/2024

Post contrast CT scan of the abdomen with special Triphasic CT study of the liver revealed:

** A reported case of breast cancer,
** In review to the previous report done outside our facility (no films available),

- The liver is mildly enlarged and it shows smooth outline and low fatty homogenous parenchymal texture with a solitary well-defined hypodense hepatic focal lesion, ?? low cystic attenuation value, seen in the right hepatic lobe segment VII, measures about 12x15mm along its axial dimensions. No mural calcifications or associated soft tissue nodules and upon contrast administration they show no CT evidence of enhancement in different phases of the study.
- No dilated biliary radicles
- Portal vein and its main branches are patent and homogeneously enhanced
- Normal CT appearance of the spleen, pancreas, both, both suprarenal glands, aorta, IVC and pelvic organs
- No pathologically enlarged abdomino-pelvic lymph nodes
- Unremarkable pelvic organs with no abnormal pelvic cystic or solid masses.
- No abdominal or pelvic ascites
- Clear scanned lung bases
- No lytic or sclerotic osseous lesions.
- Left mild sacroiliitis is seen.

Opinion:

Mild hepatomegaly with diffuse fatty deposition
Right non-enhancing hypodense hepatic focal lesion, likely simple hepatic cyst for follow-up and ultrasound correlation.

Dr. Alaa Ragab, M.Sc., FRCR

Dr. Amr Farouk, M.D.



Patient Name: Ne'ma A. Naby Ahmed Atteya	Date : 10/02/2025
Exam : MRI BRAIN WITH CONTRAST	Age : 46 Year(s)
Medical ID: 1841/2024	Pat. ID: 53908

MRI STUDY OF THE BRAIN

Technique of Examination:

- Axial T1, T2 & FLAIR WIs
- Sagittal and coronal T1WIs
- Post contrast series
- DWI

MR Findings:

- No definite enhancing intra cranial masses.
- No cerebral parenchymal areas of abnormal signal.
- Normal grey / white matter differentiation.
- Normal size and configuration of the ventricular system.
- No shift of midline structures.
- No areas of fresh blood or blood degradation product signal.
- Normal appearance of the brain stem and cerebellum.
- No extra axial collections.

Opinion:

No definite enhancing intra cranial masses.

Dr : Ahmed Elgazar

Much obliged,

.....



Patient Name: No'ma A. Naby Ahmed Attouya	Date : 04/12/2024
Exam : MAMMOGRAPHY BOTH	Age : 46 Year(s)
Medical ID: 1841/2024	Pat. ID: 53908

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

left breast pathologically proved carcinoma on NACTH.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- The breasts are Scattered fibro glandular density (ACR B)
- Regressive course of Left diffuse skin thickening with underlying trabecular thickening with UOQ focal asymmetry of high density reaching to the nipple. US showed small hypoechoic irregular nodule about 5x4mm.
- No right suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- Complementary ultrasound showed no right solid or cystic mass lesions.
- Normal right skin thickness and nipple-areola complex.
- Left no pathologically axillary lymph nodes seen in today's study.
- Right non specific axillary lymph node.

Opinion:

- Left breast pathologically proved carcinoma on NACTH with near complete radiological response (BIRADS 6)
- Right breast normal study (BIRADS 1).

Signature Dr. Nehal Hamdy

04/12/2024 3:32 PM

جمهورية مصر العربية
بطاقة تحريف الشخصية



نعمه

عبداللهى احمد عطيه

٢ ش بهنيم - ح المرجوشى - تكسيم سيدشرف
شبرا الخيمة ثمن - القليوبية

١٩٧٩/١/١٧

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KX4438545



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حاصلة على معهد فنى صناعى

أنثى مملوكة متزوجة
احمد عبدالله ابراهيم غريب

البطاقة سارية حتى ٢٠٣١/١٢/٠٨



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